



(<https://learningcenter.hfsa.org/>).



My Courses (<https://learningcenter.hfsa.org/Users/Home.aspx>)
/ **2026 AHFTC Board Certification Review - OnDemand**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=4f%2fOexzNsorKAF1lCFotvA%3d%3d>)
/ **2026 AHFTC Board Certification Question Bank**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=YSpCYks4EUa7JCy700Rkkw%3d%3d>)
/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 46 of 117



A 52-year-old male presents to his primary care doctor for recurrent episodes of syncope at the gym. TTE reveals a preserved EF, small LV cavity (LVEDD 4 cm), severe concentric LVH with posterior and septal wall thickness of 1.8 cm and an apical aneurysm. TTE was also notable for mild MR without evidence of systolic anterior motion of his mitral valve. He is referred to his local cardiologist who diagnoses him with hypertrophic cardiomyopathy (HCM) and referred him for ICD implantation given prior unexplained syncopal events and presence of apical aneurysm. He is initiated on metoprolol and continues to do well for several years.

Five years after diagnosis, he developed symptoms of palpitations and dyspnea and presents to his cardiologist after 5 days of symptoms. His JVP is elevated at 15 mmHg and he has 1+ LE edema. EKG reveals atrial fibrillation.

Which of the following anticoagulation strategies should be pursued?



A. Direct oral anticoagulant (DOAC)



B. Left atrial appendage occlusion device

C. Full dose aspirin

D. No anticoagulation is needed

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know that patients with hypertrophic cardiomyopathy (HCM) and atrial fibrillation should be anticoagulated regardless of their CHA₂DS₂-VASc score.

Correct Answer: A. Direct oral anticoagulant (DOAC)

Rationale: Data from a meta-analysis of more than 7000 patients with HCM and AF demonstrated a higher incidence of thromboembolism (27% and 3.75/100 patients, respectively) than in patients with AF without HCM. This risk was found to be independent of the CHA₂DS₂-VASc score. As such, the 2020 AHA/ACC guidelines for the diagnosis and treatment of patients with HCM

gives anticoagulation with either a direct oral anticoagulant (DOAC) or Coumadin a class I, level of evidence B–NR recommendation.

DOACs are generally preferred when no contraindication exists because of ease of use and favorable observational outcomes, though warfarin remains an acceptable alternative.

Answer B: Left atrial appendage occlusion is not considered first-line stroke prevention in HCM-associated AF and is generally reserved for patients with contraindications to long-term anticoagulation.

Answers C and D are incorrect because aspirin monotherapy does not provide adequate protection against thromboembolism, and anticoagulation is indicated in HCM patients with AF regardless of CHA₂DS₂-VASc score.

Powered by Oasis.



(<https://www.oasis-lms.com/>).